

Urethral Stricture (Treatment Choices)

WARWIKI

Patient Information · Comparing the ways to treat a narrowing of the urethra

A urethral stricture is a scar that narrows the urethra, the tube that carries urine out of the body. It slows the stream and can make it hard to empty. There are several treatments. Quick scope treatments often need repeating, and **each repeat is less likely to work**. Urethroplasty (open repair) lasts longest.

Your Choices at a Glance

Treatment	What it is	What to expect
Dilation	The scar is stretched with tubes or a balloon passed through the urethra.	Quick, usually home the same day with a catheter for 1–3 days. Best for a short first stricture under the scrotum. The narrowing often comes back.
Urethrotomy	The scar is cut open with a small blade or laser through a scope.	
Optilume	A balloon widens the scar and coats it with a medicine that slows new scar.	For a short stricture under the scrotum that came back after scope treatment. Condom for 30 days; do not father a child for 6 months.
Urethroplasty	Surgery through a cut that removes the scar and joins the ends, or widens it with a graft from inside the cheek.	Lasts longest. Catheter for about 2–3 weeks. Best for long, penile or repeat strictures, or anyone who wants the most lasting fix.
Self-dilation	You pass a smooth catheter yourself on a schedule to keep the scar stretched.	Keeps a stricture open but does not cure it. For people who cannot have, or do not want, surgery.
Perineal urethrostomy	A new urine opening is made between the scrotum and anus.	For complex strictures or repeated failed repairs. You sit to urinate.

Benefits and Risks in Numbers

more than
9 in 10

long strictures under the scrotum stayed open about 6 years after cheek-graft repair (one large center)

up to
7 in 10

needed no further procedure 3 years after Optilume for a short stricture that came back

about
half as many

needed another procedure after urethroplasty as after a repeat urethrotomy (2-year trial)

Numbers are averages from published studies. The 3-year Optilume figure may be high: some men whose narrowing came back did not have another procedure. Your own risk may differ; ask your team.

Why Repeat Scope Treatments Fall Short

- After a scope treatment has failed once, another one fails in **more than 8 in 10** men.
- Repeated cutting can make the scar longer and deeper, which makes a later repair harder.
- Guidelines advise against more than two scope treatments when urethroplasty is possible.

Learn the Terms

Stricture

A scarred, narrow part of the urethra.

Bulbar urethra

The part under the scrotum. It is the most common place for a stricture.

Penile urethra

The part inside the penis. Scope treatments rarely last here.

Graft

A patch of your own tissue, usually from inside the cheek, used to widen the urethra.

Lichen sclerosus

A skin condition that can scar the tip of the penis and urethra.

Which is right for me? It depends on where the scar is, how long it is, how many treatments you have had, what caused it, your health and what matters to you. A short first stricture under the scrotum can be treated with a scope or with urethroplasty. Long, penile or repeat strictures do best with urethroplasty.

Dilation or urethrotomy Home the same day. Catheter for 1–3 days.	Optilume Catheter for at least 2 days. Condom for 30 days.	Urethroplasty Catheter for about 2–3 weeks, often with an X-ray before it comes out.	Self-dilation Ongoing, on a schedule your team sets.
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What Causes a Stricture

Common causes are an injury to the groin or pelvis, past catheters or scope procedures, prostate surgery, infection, radiation, childhood hypospadias repair and the skin condition **lichen sclerosus**. Often no cause is found. With lichen sclerosus, repairs use a graft from the cheek, because genital skin used as a graft tends to scar again.

How Your Stricture Is Checked

- A **flow test** measures how fast you urinate, and a scan checks how well you empty.
- An **X-ray with dye** (urethrogram) or a **camera look** (cystoscopy) shows where the scar is and how long it is.
- A **urine test** checks for infection.

Questions to Ask Your Team

- Where is my stricture, and how long is it?
- What is the chance each option lasts for someone like me?
- If I have a scope treatment now, will it make a repair harder later?
- How long would I have a catheter, and when can I work and have sex?
- What are the risks to erections, ejaculation or penis length?

About Self-Dilation

Your team teaches you to pass a lubricated catheter through the narrow part on a set schedule. It can help keep the stricture open after a scope treatment, but it does not cure the scar. Some men find it hard to do, and in one survey of men doing it for years, the typical man rated his quality of life as poor. Ask how often you would need to do it.

Call your care team if you have:

- A temperature of **100.4 °F (38 °C)** or higher, or chills
- A stream that is slowing again, spraying, or a feeling that you are not emptying
- Burning or urine infections that keep coming back

Go to an emergency department if:

- You cannot urinate and your bladder feels full or painful
- Painful swelling or redness of the penis, scrotum or the area behind it, with fever
- You cannot reach the office and are worried

Three Things to Remember

1. Scope treatments are quick but often need repeating, and each repeat works less well.
2. Urethroplasty gives the longest-lasting result.
3. Tell your team if your stream slows again, even years later.